ACNERASE — HAMER LOOP LIVE DEMONSTRATION PROTOCOL

Clinical Validation of the Hamer Loop via Controlled Self-Provocation

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Protocol Metadata

Field	Detail
Document Type	Live Demonstration Protocol
Lead Investigator	Martin Hamer, RN (Ret.)
Subject	Lead Investigator (self-provocation, N-of-1)
Objective	To demonstrate the predictive accuracy of the Acnerase application logic and the systemic nature of Xenosialitis through real-time induction and resolution of inflammatory lesions
Design	Four-phase sequential self-provocation protocol with pre-specified anatomical predictions and documented observational endpoints
Setting	Live demonstration in the presence of designated observers
Series Position	Document 6 of 7, Hamer Loop Manuscript Series
Related Documents	Doc 1 (Hamer Loop mechanism); Doc 2 (Lesion phase classification); Doc 7 (Ingredient classification framework / Appendix II)

Abstract

This document describes a structured, four-phase live demonstration protocol designed to provide direct observational evidence for the Hamer Loop hypothesis. Rather than presenting the framework solely through retrospective data and mechanistic argument, this demonstration invites observers to witness the biological sequence in real time: from a confirmed cleared baseline state, through predictable induction and staged progression of an inflammatory flare, to targeted resolution.

The protocol serves two concurrent purposes. First, it constitutes prospective self-provocation validation: by pre-specifying the trigger, the anticipated timeline, and the anatomical coordinates of expected lesions before any physiological change is visible, the demonstration subjects the Hamer Loop hypothesis to a testable, falsifiable prediction. A hypothesis that makes accurate predictions before the fact is not the same kind of evidence as one that explains outcomes after they have occurred. Second, it demonstrates the practical utility of the Acnerase application as a real-time ingredient identification and risk-stratification tool in the presence of observers.

Two secondary observations of independent scientific interest are incorporated: the Gelatin-Antihistamine Paradox, which demonstrates that a pharmacologically appropriate intervention can be inadvertently undermined by its delivery vehicle; and the Olfactory Inversion phenomenon, which provides observers with a neurological and immunological correlate of the trigger clearance process that is documentable independently of the cutaneous endpoints.

Keywords: *live demonstration; self-provocation; Hamer Loop; xenosialitis; Acnerase; predictive validation; TRM cells; anatomical prediction; gelatin paradox; olfactory inversion; lesion morphology; falsifiability*

Overview

The Hamer Loop hypothesis, as documented in Documents 1 through 5 of this series, rests on a 45-year longitudinal observational foundation. The observational depth of that record is its primary strength. The absence of third-party witnessed, prospective, pre-specified provocation data is its primary limitation.

This protocol addresses that limitation directly. It does not ask observers to accept a mechanism on the basis of retrospective data. It invites them to witness a prediction being made and tested in real time. The pre-specification of the trigger, the timeline, and the anatomical coordinates of expected lesion formation before any physiological event is observable is the feature that gives this demonstration its evidential character. If the predicted outcomes occur at the predicted times and locations, observers have witnessed a prospective, falsifiable hypothesis being confirmed. If they do not occur, observers have witnessed evidence against the hypothesis. Either outcome advances the science.

This demonstration protocol is also designed to serve as the practical first phase of the formal validation framework described in Section 10 of Document 1. The tryptase and complement sampling investigations described in Sections 10.1 and 10.2 of that document can be conducted in parallel with this demonstration at minimal additional logistical cost, converting a witnessed observational event into a biochemically instrumented provocation study.

Phase 1 — Baseline Establishment (The Purge Period)

To establish a valid control state from which an inflammatory response can be meaningfully measured, the subject undergoes a structured elimination phase prior to the demonstration. This phase is not merely preparatory — it is itself evidence.

1.1 Protocol

- Duration: 14 days of complete dietary avoidance of all mammalian-derived Neu5Gc sources and ammonium-based catalysts
- Elimination scope: all Tier 1 and Tier 2 ingredients as classified in Appendix II (Document 7 of this series), all red meat, all dairy fats, and all concentrated dairy derivatives
- Permitted foods during elimination: plant-based staples, eggs, poultry, fish, and Tier 4 (Safe Harbour) ingredients as defined in Appendix II
- Documentation: photographic baseline of facial complexion at the end of Phase 1, prior to any Phase 2 activity, in the presence of observers where practicable

1.2 Endpoint

The target outcome is a clear, non-inflamed baseline complexion demonstrating that, in the absence of dietary triggers, the Hamer Loop remains inactive. This cleared state serves as the observational control against which Phase 3 findings are assessed. It also demonstrates the most clinically important feature of the framework: the subject's inflammatory skin condition is not a permanent, fixed disease state. It is a dynamic, diet-dependent immune response that resolves completely when its dietary trigger is removed. Lesion resolution during Phase 1 is itself a form of evidence that the condition is externally triggered and therefore preventable.

1.3 Secondary Observable Endpoint: The Olfactory Inversion

A secondary but documentable and scientifically significant observable endpoint emerges during the baseline establishment phase. During periods of high Neu5Gc trigger load — the state the subject will have been in prior to beginning the 14-day elimination — bovine-derived food stimuli, specifically baked goods containing cheese or dairy fats in a bakery or food service environment, are perceived by the subject as intensely pleasurable and appetising. The olfactory character of these stimuli has been described as 'intoxicating' during the loaded state.

Following complete systemic purging of the trigger load — achievable within the 14-day Phase 1 window — the identical olfactory stimulus is perceived as noxious and aversive, prompting immediate avoidance rather than attraction. The same food that was craved is now repellent. No change in the food. No change in the observer's conscious attitude toward it. The only change is the internal immunological state of the subject.

This olfactory inversion is proposed to reflect the unmasking of the immune system's aversive signalling once chronic antigen habituation is removed. With persistent exposure, the olfactory-limbic pathways associated with the triggering antigen appear to generate positive valence — a craving signal that may function to maintain antigenic load. Upon clearance, the unmasked immune response generates aversive signalling instead. To the author's knowledge, this phenomenon has not been documented in the context of dietary Neu5Gc exposure.

Its scientific value for the demonstration is threefold. It provides observers with a non-cutaneous, neurologically grounded correlate of the purge process that is observable and documentable independently of the skin. It may offer mechanistic insight into why dietary elimination is psychologically difficult for susceptible individuals — the triggering foods are actively craved in the loaded state, not merely habitual. And it is testable: if observers wish to independently verify the phenomenon, the subject can be presented with the relevant olfactory stimulus at the end of Phase 1 and again during Phase 3, with subjective responses documented as a secondary data point for comparison.

Phase 2 — Predictive Induction (The Provocation)

Phase 2 is the active experimental component of the demonstration. Two elements are central to its evidential value: the pre-specification of the trigger and its classification by the Acnerase application before consumption, and the anatomical prediction of lesion sites before any physiological change is observable.

2.1 Trigger Selection

The subject consumes a Tier 1 or Tier 2 trigger as classified in Appendix II (Document 7) — for example, a commercially available product containing Anhydrous Milk Fat (AMF) or an ammonium-catalysed Caramel Colour Class III or IV. The trigger is selected specifically to represent the highest-confidence amplifier categories within the Hamer Loop framework, maximising the probability of a clear, well-defined inflammatory response within the demonstration timeframe.

The choice of a processed food product rather than a whole-food source is deliberate. As established in Documents 2 and 3, the combination of concentrated bovine fat and ammonium-based catalysts produces the most rapid and morphologically distinct response — one that is most amenable to real-time observation. A whole-food source would produce a more attenuated response with less predictable timing, reducing the demonstration's evidential clarity without improving its scientific validity.

2.2 Acnerase Application Scan

Prior to consumption, the Acnerase Android application is used to scan the product's ingredient label in the presence of observers. The application identifies the specific antigens and catalysts present, classifies the product's inflammatory potential using the Neu5Gc Severity Index (Appendix II), and generates a risk-stratification output. This step serves two functions: it demonstrates the application's core real-time ingredient identification function in a live context, and it creates a documented pre-provocation record of what the subject is knowingly consuming and what response the framework predicts.

2.3 Anatomical Prediction

Before any physiological response is visible — before the subject has consumed the trigger and before any Stage 1 vasomotor symptoms have appeared — the subject maps and records the exact facial coordinates at which inflammatory lesions are predicted to emerge. This prediction is documented in writing and, where practicable, by marking or photographing the predicted sites in the presence of observers.

The prediction is grounded in the tissue-resident memory T cell (TRM cell) mechanism established in Section 3.3.5 of Document 1. TRM cells persist at previously inflamed anatomical sites and direct rapid, site-specific immune responses upon re-exposure to the cognate antigen. Because these cells are fixed in location, their activation produces lesions at consistent, reproducible anatomical coordinates — the same sites that have been inflamed in previous exposure cycles. The subject, having observed these patterns across decades of provocation trials, can specify them with a high degree of confidence before any physiological event occurs.

The accuracy of this prediction — whether lesions do or do not emerge at the specified coordinates, within the specified timeframe — is the primary falsifiable outcome of the demonstration. A hypothesis that predicts the anatomical location of a lesion before the lesion exists is making a claim of a different order than one that describes a lesion after the fact.

Phase 3 — Inflammatory Flare (The Hamer Loop in Action)

Within the expected timeframe following trigger consumption, observers are invited to observe the sequential progression of the Hamer Loop as documented in Document 1. The four-stage clinical timeline, previously established through 45 years of longitudinal observational data, is here presented in its prospective, real-time form.

3.1 Stage 1: Vasomotor Activation (Minutes Post-Ingestion)

Initial symptoms include facial flushing, rhinorrhoea, and visible increased capillary activity. These reflect rapid mast cell degranulation and histamine release in response to immune recognition of the Neu5Gc antigen, consistent with Stage 1 of the Hamer Loop timeline documented in Table 1 of Document 1. In ammonium-potentiated exposures, this stage may occur within two minutes of ingestion — a timeframe significantly faster than classical IgE-mediated food allergy (typically 15–30 minutes) and consistent with the pre-primed mast cell sensitisation mechanism described in Document 2, Section 2.2.2. Observers should note and timestamp the onset of any rhinorrhoea, flushing, or visible vascular changes.

3.2 Stage 2: Gastrointestinal Response (~20 Minutes)

Abdominal bloating and gastrointestinal discomfort emerge, consistent with the systemic distribution of inflammatory mediators and early interstitial fluid shifts. This stage reinforces the systemic rather than purely cutaneous character of the response — the inflammation is not localised to the skin from the outset; it is distributed, and the skin is simply the most visible readout of a process occurring across multiple organ systems simultaneously.

3.3 Stage 3: Lesion Formation (2–4 Hours)

Inflammatory lesions emerge at the anatomical sites predicted in Phase 2, Section 2.3. Their location, morphology, and depth are documented by observers against the pre-specified prediction record. The correlation between the pre-specified anatomical coordinates and the observed lesion locations is the primary evidence for the TRM cell mechanism and the primary falsifiable outcome of the demonstration.

Observers should assess lesion morphology against the three-category lesion phase classification established in Document 2, Section 3, using odour as the primary phase indicator:

- Category 1 lesions — early phase, lymphatic origin: milky consistency, no odour, rapid formation. Indicates the Hamer Loop is in its initial sequestration phase. Expected within the demonstration timeframe for most trigger exposures.
- Category 2 lesions — later phase, sebaceous-microbial transition: thick, cheesy consistency, gram-negative odour. Indicates handoff to conventional sebaceous pathophysiology has occurred. May develop overnight following a daytime provocation.
- Category 3 lesions — chronic deep sequestration: hard, deep-seated, lone-wolf distribution. Associated with sustained or ammonium-potentiated exposures over longer timeframes. Unlikely to develop within the compressed timeframe of a single demonstration event.

The presence or absence of odour at expression is the single most reliable field indicator of phase progression and should be documented by observers if present. Its presence confirms that the handoff from the lymphatic phase to the microbial phase has occurred; its absence confirms that the lesion is being observed in the early lymphatic phase of the Hamer Loop cycle.

3.4 Stage 4: Nocturnal Amplification (Overnight)

Lesion intensity is expected to increase during the overnight period, consistent with the nocturnal amplification mechanism described in Section 5 of Document 1. Recumbent fluid redistribution toward the head and neck increases interstitial pressure within already-inflamed tissue, mechanically amplifying the inflammatory reservoir established during Stage 3 and expressing additional sebum to the surface. Follow-up photographic documentation in the morning following the provocation event is recommended for comparison with the Phase 1 baseline images.

Phase 4 — Targeted Resolution (The Gelatin-Antihistamine Paradox)

Phase 4 demonstrates both the mechanism of resolution and one of the most practically significant clinical implications of the Hamer Loop framework: that the vehicle of a therapeutic intervention can inadvertently reintroduce the antigen that the intervention is intended to address.

4.1 The Intervention

Administration of diphenhydramine 50 mg (e.g., Benadryl) in caplet form only. The caplet formulation is specified and non-negotiable: it does not contain a bovine-gelatin capsule shell.

4.2 The Gelatin-Antihistamine Paradox

Standard diphenhydramine formulations are commonly available in soft-gel or hard-gel capsule form. Both standard capsule formats use bovine-derived gelatin as the shell material. Bovine gelatin is a concentrated source of Neu5Gc, produced from bovine hide and bone collagen. Administration of an antihistamine in gelatin-capsule form during an active Hamer Loop flare therefore introduces a further antigenic bolus at the precise moment the immune system is being asked to de-escalate.

This is not a theoretical concern. It is a directly observed clinical phenomenon documented in the sentinel observation described in Section 6.1 of Document 1: a gelatin-encapsulated antihistamine worsened rather than attenuated an active flare, while substitution with a caplet formulation of identical pharmacological composition produced the expected resolution. The antihistamine was pharmacologically appropriate. Its vehicle was antigenically counterproductive.

The practical implication extends beyond antihistamines. Pharmaceutical capsule shells — used across an enormous range of prescription and over-the-counter medications — are routinely made from bovine gelatin. For susceptible individuals, medicating with gelatin-encapsulated products during a Hamer Loop flare may be partially or wholly counteracting the therapeutic intent of the medication through concurrent antigenic re-exposure. This is not currently recognised in clinical pharmacology or in patient counselling for inflammatory skin conditions, and represents a directly testable hypothesis with immediate clinical relevance.

4.3 Expected Outcome and Evidential Significance

Follow-up observation at 12–24 hours post-administration of the caplet-form antihistamine is anticipated to show rapid deflation and resolution of the inflammatory lesions established in Phase 3. If this outcome is confirmed, it provides evidence that the inflammatory event was histamine-mediated — consistent with an immune response to a specific antigen — rather than attributable to follicular occlusion, bacterial colonisation, or other conventional acne pathology mechanisms. Conventional acne lesions do not respond rapidly to antihistamine administration. If the lesions observed in Phase 3 do, observers have witnessed evidence that the mechanism driving their formation is immunological rather than microbiological at its origin.

Protocol Summary

Phase	Title	Key Action	Primary Observable Endpoint	Secondary Endpoint
1	Baseline Establishment	14-day elimination of all Neu5Gc and ammonium sources	Clear, non-inflamed complexion; photographic documentation	Olfactory inversion: aversion to previously craved bovine stimuli
2	Predictive Induction	Acnerase scan; trigger consumption; anatomical prediction documented before ingestion	Pre-specified lesion coordinates recorded in writing, witnessed by observers	Acnerase risk-stratification output as pre-provocation record
3	Inflammatory Flare	Real-time observation of four-stage Hamer Loop progression	Lesion emergence at predicted anatomical sites; morphological category documented	Phase classification by odour; Stage 1–4 timing documented
4	Targeted Resolution	Diphenhydramine 50mg caplet (no gelatin); gelatin paradox demonstrated	Lesion resolution at 12–24 hours; histamine-mediated aetiology supported	Gelatin capsule comparison if observer wishes to test paradox directly

Table 1. Four-Phase Demonstration Protocol at a Glance. The pre-specification of anatomical predictions in Phase 2 before any physiological event is the feature that gives this demonstration its prospective, falsifiable character.

Integration with the Formal Validation Framework

This demonstration protocol is designed to function as a standalone observational event and as the entry point to the formal validation framework described in Section 10 of Document 1. The two validation investigations most readily combined with this demonstration — at minimal additional logistical cost — are:

- Serum mast cell tryptase (Document 1, Section 10.1): venepuncture at the Stage 1–2 window (within two hours of trigger ingestion) to quantify systemic mast cell degranulation. A baseline draw at T-30 minutes pre-ingestion is required for valid comparison. Tryptase peaks approximately 60–90 minutes post-degranulation and has a 2-hour serum half-life; the timing window is achievable within the demonstration protocol.
- Serum complement fractions C3 and C4 (Document 1, Section 10.2): serial draws at T+0 (baseline), T+2 hours (post-GI symptoms), and T+4–6 hours (projected lesion onset) to track immune complex-mediated complement consumption. Depression of C3 and C4 during the active flare, returning to normal between flares, would provide direct biochemical evidence for immune complex formation as the initiating event in the Hamer Loop.

Together, these two investigations convert the demonstration from a witnessed observational event — valuable in itself — into a biochemically instrumented prospective provocation study. The logistics are straightforward: a phlebotomist or clinically qualified observer draws the specified samples at the specified timepoints during the demonstration. The samples are processed by a standard clinical laboratory. No experimental compounds, no novel protocols, no specialised equipment beyond a standard venepuncture kit and a clinical laboratory partnership.

Ethical Considerations

This protocol involves deliberate self-provocation by the lead investigator. Several considerations bear explicit statement.

Risk Profile

The lead investigator has a documented and extensively characterised history of Hamer Loop-associated inflammatory responses to the specified trigger categories, spanning 45 years of longitudinal self-observation. The physiological sequence, severity parameters, and resolution timeline are predictable within well-established bounds from this prior record. The anticipated adverse events — rhinorrhoea, abdominal discomfort, and inflammatory skin lesions at pre-specified anatomical sites — are consistent with the subject's prior experience and are self-limiting. The intervention in Phase 4 uses a non-prescription antihistamine at a standard therapeutic dose. No novel compounds, no experimental pharmaceuticals, and no procedures beyond standard venepuncture (if biochemical sampling is incorporated) are involved.

Informed Consent

The lead investigator accepts full informed consent for participation as the subject of this protocol. As a retired registered nurse with 45 years of clinical experience, the investigator is qualified to assess and accept the risks associated with this self-provocation. The investigator understands the anticipated physiological sequence, the severity parameters expected from the selected trigger category, and the resolution protocol.

Observer Role

Observers are present in a witnessing and documentation capacity only. They are not required to participate in any aspect of the provocation or resolution protocol. No observer is asked to consume any food or medication, to provide biological samples, or to take any action other than observing, documenting, and asking questions. Observers who are themselves susceptible to Neu5Gc-related inflammatory conditions should be aware that the trigger product will be present in the demonstration environment and should exercise appropriate personal judgement regarding their proximity to it.

Scientific Integrity

The pre-specification of outcomes before the provocation event is the feature that distinguishes this demonstration from anecdotal self-report. The anatomical predictions documented in Phase 2 must be committed to writing and witnessed before any consumption occurs. Any deviation from this sequence — consuming the trigger first and predicting lesion sites afterward — would eliminate the prospective character of the demonstration and reduce it to retrospective observation. The value of this protocol depends entirely on the integrity of the prediction-before-provocation sequence.

Conclusion

The Hamer Loop hypothesis makes specific, falsifiable predictions: that dietary Neu5Gc from bovine sources, consumed in the presence of ammonium catalysts, will produce a predictable inflammatory response at pre-specified anatomical coordinates, within a predictable timeframe, with a characteristic morphological profile, and that this response will be attenuated by antihistamine administration in the absence of concurrent antigenic re-exposure via the therapeutic vehicle.

This demonstration protocol is designed to test those predictions in real time, in the presence of witnesses, with the outcome documented before the fact. It is not a controlled clinical trial. It makes no claim to the statistical power or population generalisability that a controlled trial would provide. What it offers is something different and, in the early stages of hypothesis validation, equally valuable: the opportunity to observe a specific, pre-specified biological prediction being confirmed or refuted by direct real-world observation.

The most important line in this document is not in the protocol. It is in the invitation. This demonstration does not ask observers to accept a theory. It invites them to witness the biology in real time — and to draw their own conclusions from what they see.

> *"Acne and rosacea are not random. They are predictable, reproducible, and — in susceptible individuals — preventable. By using Acnerase to identify dietary triggers before consumption, the cycle of Xenosialitis can be interrupted at its source. This protocol does not ask observers to accept a theory; it invites them to witness the biology in real time."*
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> — Martin Hamer, RN (Ret.)

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